Give Maria her judgment back: one trustworthy, prioritised morning instead of seven screens

A Case Management Workbench MVP for a health plan's RN and social-work case managers, built on HiLabs-standardised data

THE PROBLEM

~79% of a case manager's day goes to hunting for data, dialling numbers that fail and re-typing documentation (our model, slide 3)

2.5 real clinical conversations a day, the only part that uses her ICU judgment

~50% of patients leave hospital with an unintended medication discrepancy (systematic review)

12–34% of first follow-up visits have the discharge summary available (Kripalani, JAMA)

WHAT WE BUILD (MVP: 3 screens, overlay on the plan's existing CM system)

Today queue: who needs me, and why

Ranked by clinical risk, regulatory clock (TRC 2-day, FUH 7-day), impactability and reachability. Exactly three reasons per member and a plan for the day built around each member's best call time.

Member 360: one screen, every source

An AI 'since last contact' brief where every line cites its source and date, freshness shown per source, and conflicts shown side by side, never auto-resolved. Medication diff across plan list, hospital and pharmacy.

Outreach & notes: talk, don't type

Contact ladder with validated numbers and consent, an assessment that is about 60% pre-filled from data she verifies, and an AI-drafted note that becomes the contact note, care plan and audit record. The clinician signs.

90-DAY PILOT TARGETS

21 → 5 min prep per member

2.5 → 5 real conversations / CM / day

–40% documentation time

≥ 85% discharges contacted in 2 business days

QR code to the live prototype (added after Vercel deploy)

Try it: the clickable prototype has 5 synthetic members and a Monday 08:00 snapshot

Why HiLabs wins: incumbents (Jiva, GuidingCare, Epic) digitise the form and leave data quality to the nurse. HiLabs already cleans payer data at scale (MCheck, provider accuracy, outreach AI), so the facts arrive trusted, cited and current, and the form fills itself. AI drafts; the clinician decides.

Maria is an ICU-grade clinician working as a data clerk: two real conversations by 3 pm

Persona (case manager only) and a Monday in her life. Composite built from the case, job postings at large MA and Medicaid plans, and published workflow studies

A. PERSONA: MARIA R., RN, CCM



Remote telephonic complex CM + TOC nurse

Regional MA, D-SNP and Medicaid plan · 20 yrs ICU · licensed in 3 compact states · 48 active cases

Goals	Catch the member who is about to crash. Pass the NCQA/state file audit. Go home on time.
Superpower	Pattern-spotting: 'this med list doesn't add up', 'the caregiver is drowning'.
Tools today	CM system, claims portal, UM/auth, PBM, HIE portal, old notes, her Excel tracker: 6–7 screens per member.
Frustrations	Alerts with no reason · stale claims · numbers that don't work · the same facts typed 3–4 times · no idea who needs her first.
Constraints	Licensure by member state · consent rules (HIPAA, 42 CFR Part 2, TCPA) · audit-ready notes for every attempt.
Supporting cast	Social worker, BH counsellor (LCSW), CHW, UM nurse, supervisor. Not personas here, but the routing must know them.

B. MONDAY, 08:00 → 19:00 (A DAY IN THE LIFE, CASE NARRATIVE + OUR RESEARCH)

08:00 6 discharge alerts since Friday. None say why the member was admitted.

Untimely data

08:20 Member #1: opens 7 screens. Latest claim is 45 days old.

Data hunting

09:05 Hospital list says 'stop lisinopril'; pharmacy refilled it. She calls the hospital to check.

Messy data

10:00 4 calls: 3 voicemails, 1 disconnected number.

Hard outreach

11:30 Reaches 1 member; must complete a 40-field assessment before closing the contact.

Documentation

13:00 Hunts for a discharge summary that has not arrived.

Untimely data

14:30 2nd real conversation. Senses the caregiver is overwhelmed, but there is no field for that.

Judgment unused

17:30 Re-types the same facts into the care plan and audit forms, after hours.

Documentation

C. WHAT A BAD DAY COSTS

Member

Missed 2-business-day contact after an HF discharge; a med conflict goes unseen.

Plan

One avoidable readmission ≈ $15.2k (HCUP); TRC/FUH misses hit HEDIS and Star ratings.

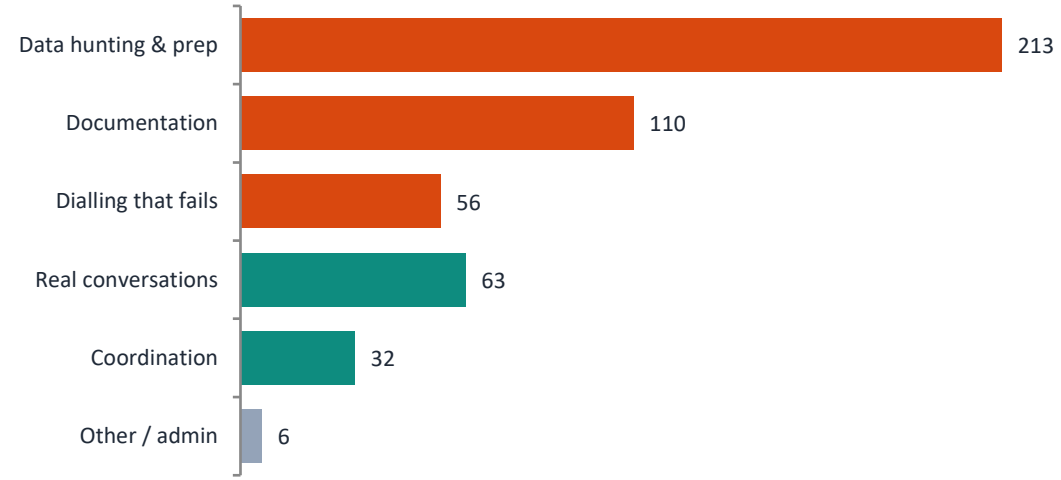
Maria

58% of RNs report burnout most days (AMN 2025); 77% of clinicians do admin after hours (AMIA).

79% of Maria's day is hunting, failed dialling and re-typing; only 13% is clinical talk

A transparent model: every input is an editable assumption in the annex (Day_Lost_Model tab). Confidence: medium, to be validated with a 2-week time-motion baseline in the pilot

A. WHERE THE DAY GOES (MINUTES PER 8-HOUR DAY)



Bucket	Arithmetic (assumption)	Min	Share
Hunting & prep	10 members × (6.5 screens × 2.5 min + 5 min reconciling)	213	44%
Documentation	1 assessment × 35 + 10 notes × 5 + 25 re-entry	110	23%
Failed dialling	18 attempts × 78% no-connect × 4 min	56	12%
Conversations	2.5 × 25 min (case: '2 by 3 pm')	63	13%
Coordination + other	4 tasks × 8 min + remainder	38	8%

Connect rate 22% (industry 20–30%; Medicaid reach can be <20%). Screens per member from the case.

B. PAIN POINTS RANKED BY IMPACT

Impact = time cost (1–5) × member risk (1–5) × how far a workbench can fix it (1–3)

#	Pain point (obstacle in the case)	Evidence	T	R	F	Score
1	Data hunting across 6–7 systems; sources disagree (messy data)	213 min/day; ~50% of discharges have a med discrepancy	5	5	3	75
2	Same facts re-typed into assessment, care plan, notes and audit forms (documentation)	110 min/day; after-hours charting	4	3	3	36
3	No ranking and hidden clocks: 'who needs me today?' (work burden)	TRC 2-day and FUH 7-day windows missed silently	2	5	3	30
4	Late or partial data: ADT without a reason, summaries missing (untimely data)	Diagnosis optional in ADT rules; summary at 12–34% of first visits	3	5	2	30
5	Wrong numbers, wrong times, consent and trust barriers (hard outreach)	≥ 24% of Medicaid numbers inaccurate (NBER); 78% of calls fail	3	4	2	24

So what: fix the facts first. Hunting and documentation are 2/3 of the lost time, and both collapse once facts arrive pre-assembled with sources. Ranking and clocks cost few minutes but carry the most member risk, so they ship in the same MVP. Outreach gets a data fix (valid number, best time) before any new channel.

15 must-have datasets, each tied to a decision; claims are the slowest and least useful

Inventory: source, what it tells the clinician, freshness, expected quality problems and how the screen handles them, and priority

Dataset	Source / standard	What Maria decides with it	Fresh	Quality problem → how the screen handles it
ADT event alerts	HL7 v2 A01/A03/A04 via HIE (CMS CoP)	Event now: starts TRC/FUH clocks, re-ranks queue	**Minutes**	No diagnosis (optional), late batches, name typos → 'reason unknown' chip + CCD request
Discharge summary / CCD	C-CDA / FHIR; Provider Access API (2027)	Why admitted, discharge meds, follow-up plan	**Hours–never**	Partial or missing sections → shown as 'not provided', never silently blank
Pharmacy fills + rejects	NCPDP via PBM; NDC → RxNorm	What was actually picked up; gaps; blocked new drugs	**1–3 days**	Cash fills invisible, rejects hidden → rejects on timeline; gap = 'unknown', not 'non-adherent'
Med list diff (derived)	Plan list × discharge × fills	Reconcile before harm (TRC med rec)	**On change**	3 lists disagree → side-by-side diff; AI proposes, clinician resolves
Labs & observations	HL7 ORU / FHIR; LOINC	Control and safety: A1c, NT-proBNP, K+, eGFR, PHQ-9	**1–3 days**	Duplicate feeds, mixed units → normalised, de-duplicated, trend + age badge
Prior auth / UM	Plan UM; CMS-0057 PA API	Is a needed drug blocked? What unblocks it?	**Real-time**	Siloed from CM → PA status inline on the med + 'evidence available' link
Contact history + call centre	CM system, dialler, CRM	What was tried; signals buried in member-services calls	**Real-time**	Unrouted inbound notes → AI tags cost / caregiver-strain signals for review
Contact-point quality	Carrier lookup, reassigned-number DB, ADT	Which number first, when, and is it legal to dial	**Weekly + events**	≥ 20–24% wrong numbers → confidence per number; reassigned numbers suppressed
Care gaps & clocks	HEDIS engine (TRC, FUH, AMR), SNP MOC	What is due when, which drives urgency	**Daily**	Gap closure lags → countdown on the queue row
Provider directory	HiLabs MCheck; NPPES	Is the PCP real, active, accepting? Next slot?	**Weekly**	Directories inaccurate → inactive PCP flagged + in-group alternative
Member & eligibility	834 enrollment, plan MDM	Who, PCP, language, dual status, who she may speak with	**Daily**	Stale PCP/address, duplicate IDs → identity resolution, merge request
Care plan & notes	CM system	Goals, barriers, last conversation	**Real-time**	Long free text, duplicate cases → cited 'since last contact' brief
SDOH screens & HRA	AHC-HRSN / PRAPARE, D-SNP HRA, Z codes	Barriers: housing, food, transport, phone, caregiver	**8–12 months**	Stale; Z codes on only 1–1.6% of claims → age badge; newer source wins
Medical claims	837I/P: ICD-10-CM, CPT/HCPCS, DRG	History and utilisation pattern	**30–60+ days**	Lag hides recent events → 'complete through <date>'; absence ≠ negative
Risk scores	LACE, plan ML (input only)	Long-run risk, one of 4 ranking inputs	**Monthly**	Stale, opaque → drivers shown, not raw scores

Phase 2: appointments feed · BH/SUD records (42 CFR Part 2, consent-gated) · remote monitoring · closed-loop community referrals · supplemental benefits (rides, meals, phone) · OASIS/MDS · PDMP. **Deliberately out:** HCC/risk-adjustment coding prompts (conflict with clinical trust).

Insight lives in combinations: 2–4 datasets together tell Maria what no single screen can

Six recipes from the synthetic patients, the trust rules every fact follows on screen, and our answer on SNOMED CT as an 'umbrella'

A. DATASETS COMBINE INTO A DECISION

Datasets combined	Insight shown on screen	What Maria does differently
ADT discharge · Discharge CCD · Pharmacy rejects · Plan med list	New HF drug not dispensed (PA pending); old ACE-inhibitor still at home; diuretic dose doubled	Calls today; 3-way med rec with the 36 h washout rule
Provider dir. (MCheck) · Discharge plan · Appointments	PCP on file has left; in-group PCP has a slot inside the 7-day window	Books the right doctor first time
Labs ×2 feeds · Fills + rejects · Call-centre log	A1c 7.4 → 9.8% since refills ran out and the formulary dropped her drug	Opens with access, not an adherence lecture
ADT (fuzzy match) · Fills (6 rescue, 0 controller) · Carrier lookup	Uncontrolled asthma; old number reassigned; new mobile from Sunday's ED visit	Stops wasted dials; uses the new number + pharmacist
ADT psych discharge · FUH clock · Fills · Discharge meds	Buprenorphine runs out Wed and is missing from the discharge list; FUH closes Thu	Prescriber bridge + telehealth before Thu
ED CCD · Labs (eGFR 22) · 2nd pharmacy fill	NSAID prescribed with CKD 4 and K+ 5.4	Calls the prescriber before the next dose

B. TRUST RULES: EVERY FACT ON SCREEN

Provenance	value + source + event date + date received
Freshness	pill per source: green days · amber weeks · red lagged
Conflict	both sides shown in amber; never auto-resolved
Confidence	dot: identity match %, phone validity, self-report age
Absence ≠ no	'claims complete through 18 Sep', not 'no admissions'

C. CODES → CLINICAL MEANING (SNOMED VERDICT)

Source	Code (system)
Claim	I50.23 (ICD-10-CM)
Discharge CCD	443253003 (SNOMED CT)
Pharmacy	1656340 (RxNorm) ← NDC 00904758004
Lab	33762-6 NT-proBNP (LOINC)

Maria sees: 'Heart failure, worsening · LVEF 30% · new Entresto not dispensed'

SNOMED CT is not one umbrella: CPT has no maintained map, and US drugs run on RxNorm. Use one standard per domain (SNOMED conditions · RxNorm drugs · LOINC labs, as OMOP does), keep the source code, and show plain language.

Five synthetic members, five different ways the data lies, each with an 8 am Monday answer

Annex: 16 datasets (10–16 per member), 143 records, amber-flagged planted imperfections; every ICD-10-CM, LOINC and RxNorm/NDC code verified against NLM APIs

#1 · 94

Harold Brennan, 71

MA HMO · TOC

Fresh heart-failure discharge

PLANTED IMPERFECTIONS

- ADT admit had no diagnosis
- PCP on file left practice (834 stale)
- 3 med lists disagree
- Claims 52 days behind

8 AM MONDAY

Call this morning (TRC clock Tue). Reconcile lisinopril / Entresto / furosemide, send LVEF to the PA, book Dr. Nair Thu.

#2 · 91

Tanya Brooks, 33

Medicaid · BH CM

BH discharge + housing loss

PLANTED IMPERFECTIONS

- Discharge list omits buprenorphine
- SDOH 'housed' vs shelter on ADT
- Prepaid phone out of minutes
- Part 2 SUD records restricted

8 AM MONDAY

Shelter-desk callback (no PHI). Telehealth Tue (FUH Thu). Bridge buprenorphine; loop in the LCSW.

#3 · 86

Thi Mai Tran, 82

D-SNP dual · MOC

2nd fall · caregiver strain

PLANTED IMPERFECTIONS

- Duplicate Medicaid ID + open LTSS case
- NSAID with CKD 4 at a 2nd pharmacy
- Caregiver strain only in call log
- ICD code sent without decimal

8 AM MONDAY

Stop ibuprofen via prescriber now. Merge record with Dana P. Call daughter after 17:00. Post-fall HRA.

#4 · 78

Tyler Kowalski, 29

Medicaid · Complex CM

Unreachable frequent ED user

PLANTED IMPERFECTIONS

- Name/DOB differ across hospitals (92%)
- Phone on file reassigned
- Claims show 2 of 4 ED visits
- Controller prescribed, never filled

8 AM MONDAY

Suppress old number. SMS + call the ED-captured mobile at noon. Pharmacist note at QuickRx. PCP Wed.

#5 · 61

Denise Carter, 54

Medicaid · Diabetes

Rising risk; stopped refills

PLANTED IMPERFECTIONS

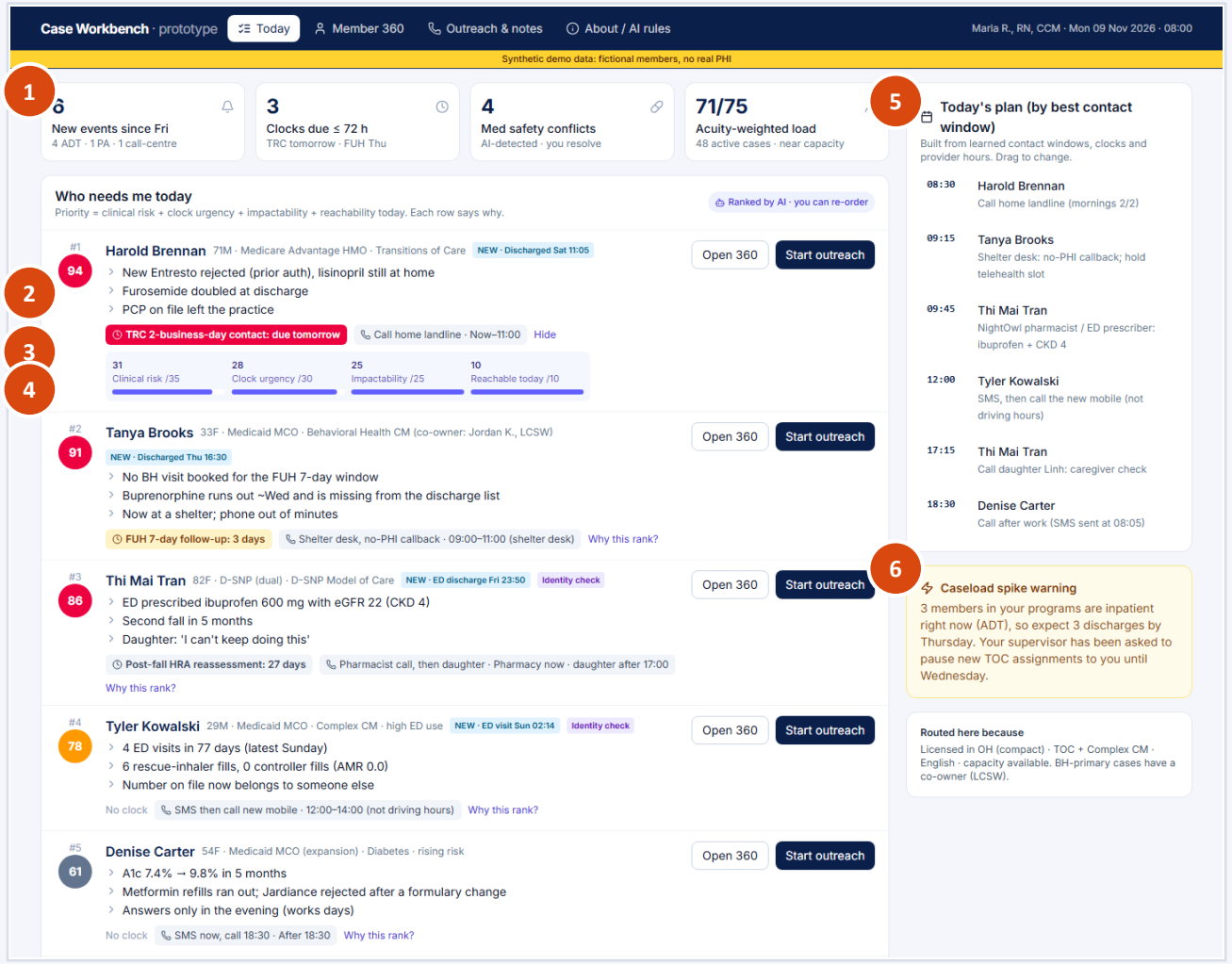
- Same A1c in 2 units (2 feeds)
- Rejected Jardiance hidden on paid claims
- Cost call never routed to CM
- Risk model still says 'low'

8 AM MONDAY

Not urgent today. SMS now, call 18:30. Metformin renewal + preferred SGLT2i. Thu evening clinic.

Screen 1, Today queue: 'who needs me, and why' answered before 08:05

Above the fold: only what changes Maria's next hour. Ranked, explained, clock-aware, and timed to when each member actually answers

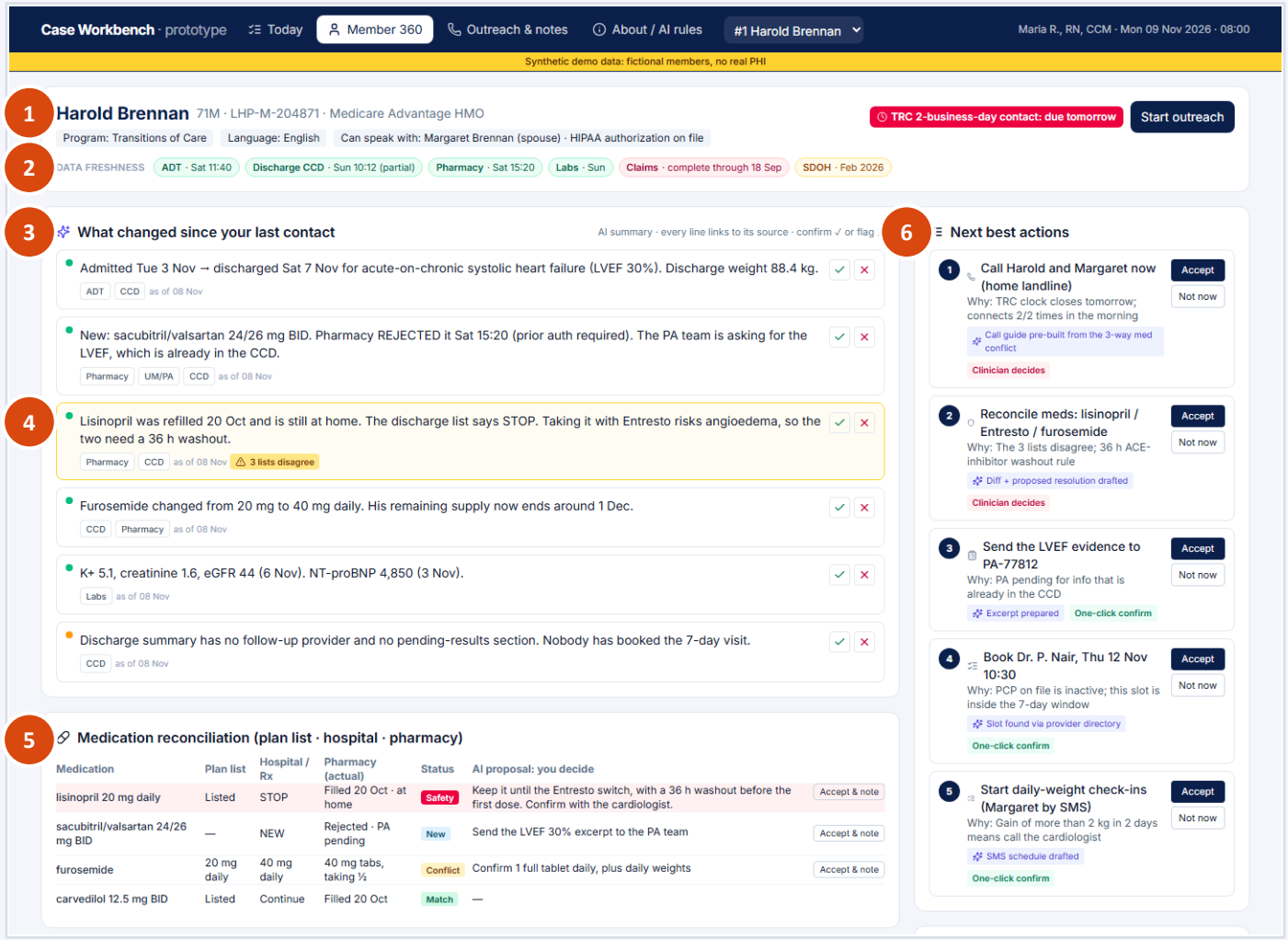


WHAT SITS WHERE, AND WHY

1. **Overnight delta:** 6 new events, 3 clocks ≤ 72 h, 4 med-safety conflicts, acuity-weighted load (71/75), not raw case counts
2. **Ranked rows, 3 reasons each:** No unexplained scores. Each reason is a fact Maria can verify in one click
3. **Regulatory clocks:** TRC 2-business-day contact, FUH 7-day, post-fall HRA shown as a countdown on the row
4. **'Why this rank?':** Priority = clinical risk /35 + clock urgency /30 + impactability /25 + reachable today /10. She can override; overrides train the model
5. **Today's plan:** Calls slotted into each member's learned answer window (Harold mornings, Denise 18:30)
6. **Spike warning:** ADT shows 3 members inpatient now, so the supervisor pauses new TOC assignments before the wave hits

Screen 2, Member 360: the 7 screens become 1, with sources, freshness and conflicts visible

Walk-through: Harold Brennan, 2 days post HF discharge. Everything Maria needs to call him with confidence, in about 2 minutes of reading

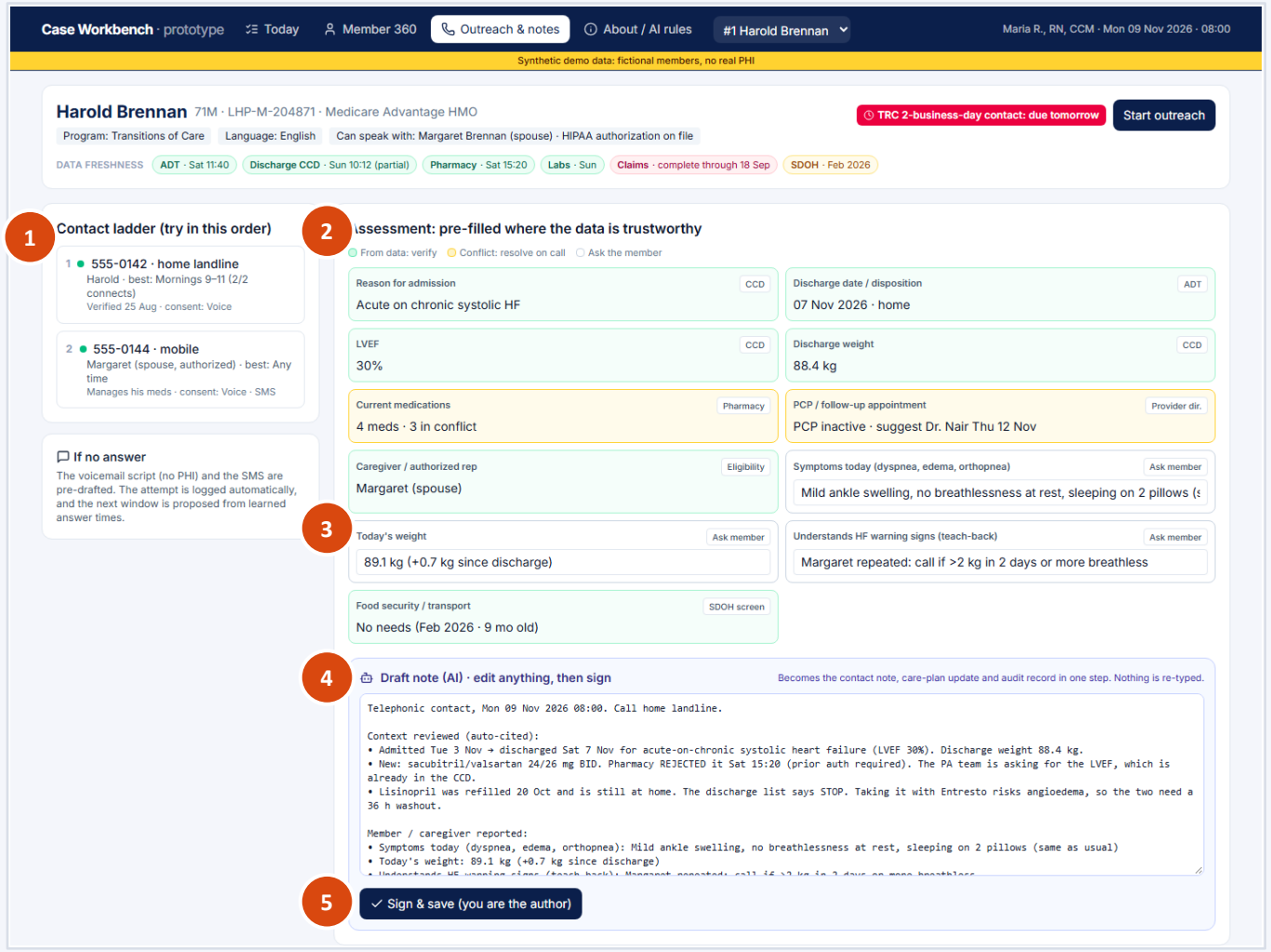


WHAT SITS WHERE, AND WHY

1. **Header:** Who she may speak with (HIPAA authorisation), language, active clock and one-click outreach
2. **Freshness strip:** One pill per source. Claims are red ('complete through 18 Sep'), so she knows why the admission isn't on them
3. **Since-last-contact brief (AI):** Every line cites sources and dates, with a confidence dot. ✓ confirm or ✗ flag; flags go to the HiLabs data team
4. **Conflict cards:** Amber when sources disagree ('3 lists disagree'); both sides are shown and nothing is auto-resolved
5. **Medication diff:** Plan list vs. hospital vs. pharmacy. AI proposes a resolution, the clinician decides and documents
6. **Next best actions:** Ranked, each with a why, an AI-prepared artifact and an owner tag: clinician decides / one-click confirm / automated

Screen 3, Outreach & notes: Maria talks; the system drafts, she verifies and signs

Walk-through: Harold's call. 8 of 11 assessment fields pre-filled from data; one AI draft replaces four rounds of re-typing



WHAT SITS WHERE, AND WHY

1. **Contact ladder:** Numbers ranked by confidence, owner, learned best window and consent; reassigned numbers suppressed (TCPA)
2. **Pre-filled assessment:** Green = from data (verify), amber = conflict to resolve on the call, white = ask the member
3. **Ask only what data can't tell:** Symptoms, today's weight, teach-back: the judgment questions
4. **One draft, many outputs:** The note becomes contact note + care-plan update + audit evidence (TRC/FUH), so nothing is re-typed
5. **Clinician signs:** Maria is the author. Edits and unedited acceptance are logged as guardrails

AI CONTRACT: WHAT IS AUTOMATED VS CONFIRMED

Automated: de-dup, unit/code normalisation, freshness, number validation, call logging

AI drafts → you confirm: brief, ranking, med diff, pre-fill, note, tasks, PA evidence

Clinician only: med changes, escalation, safety assessment, goals, merges < 95%, closure

Three members, three screens: 135 minutes of hunting become 38 of care

Before/after minutes are our estimates from the day model (prep + dialling + documentation per member), to be measured in the pilot

	1 · Today queue shows	2 · Member 360 reveals	3 · Outreach & notes	4 · Outcome, documented once	Min before → after
Harold · HF discharge	#1 (94): TRC contact due tomorrow; Entresto rejected; PCP left	Brief: PA needs LVEF (in CCD); lisinopril at home; furosemide doubled. Med diff marks a safety conflict	08:30 landline (2/2 morning connects); 8/11 fields pre-filled; teach-back with Margaret	Med rec signed; LVEF sent to PA; Dr. Nair Thu booked; TRC engagement + med-rec evidence captured	**45 → 12**
Tanya · BH + housing	#2 (91): FUH closes Thu; buprenorphine missing; phone out of minutes	Part 2 banner (consent needed); address conflict → shelter; med diff: buprenorphine ABSENT	Shelter-desk callback, no PHI; SMS Wi-Fi telehealth link; C-SSRS safety check on the call	Telehealth Tue 14:00 (FUH met); prescriber bridge requested; LCSW co-owner notified; phone + housing referrals	**60 → 16**
Tyler · frequent ED	#4 (78): 4 ED / 77 days; 'identity check' chip	92% match across hospitals → confirm; AMR 0.0; old number reassigned; new mobile from ED	Old number suppressed; SMS then call 555-0193 at noon; pharmacist note at QuickRx	Identity + SMS consent captured; $0 controller filled; PCP Wed; ED pattern on watch	**30 → 10**

Ship the trust layer first: queue + 360 + med diff; measure conversations, not clicks

RICE prioritisation, baseline → target metrics, and the risks that would kill clinician trust, with mitigations

A. MVP VS LATER (RICE = R × I × C ÷ E)

Feature	R	I	C	E	Score	
Ranked queue + clocks	5	4	0.8	2	8.0	MVP
Member 360: freshness + conflicts	5	5	0.9	3	7.5	MVP
Cited 'since last contact' brief	5	4	0.7	2	7.0	MVP
Contact ladder + number validation	4	4	0.8	2	6.4	MVP
Medication diff (3 lists)	4	5	0.8	3	5.3	MVP
Assessment pre-fill + note draft	5	4	0.7	3	4.7	MVP
Supervisor rebalancing view	2	3	0.6	3	1.2	Later
Scheduling write-back	3	3	0.5	4	1.1	Later
Ambient call transcription	4	4	0.5	4	2.0	Later
Deterioration prediction ML	3	3	0.4	5	0.7	Later

Deliberately out: AI messaging members unsupervised; AI clinical decisions; HCC coding prompts; replacing the CM system of record (we overlay it).

B. METRICS: BASELINE → 90-DAY TARGET

Metric	Baseline	Target
North star: real conversations / CM / day	2.5	5
Prep time per member	~21 min	≤ 5 min
Documentation per contact	~11 min	−40%
Notes finished after 17:30	measure wk 0–2	halve
Outreach connect rate	~22%	+10 pp
Discharge contact ≤ 2 business days	~50% (assumed)	≥ 85%
FUH 7-day / TRC med rec	plan baseline	+10 / +15 pp
Active cases per CM (acuity-wtd)	48	55–60
30-day readmits (TOC cohort, lagging)	~17%	−1–2 pp
Guardrails: AI drafts accepted unedited	—	watch > 90%
Alerts dismissed as not useful	—	< 15%

C. RISKS → MITIGATIONS

Wrong or stale data

Provenance + freshness on every fact; conflicts never auto-resolved; ✗ flags route to the HiLabs data team

Alert fatigue

Ranking instead of alarms; max 3 reasons per row; dismiss reasons logged; target < 15% dismissals

Trust & automation bias

Citations per line; confirm per item; track unedited acceptance; clinician is always the author

Privacy & consent

Part 2 gating, HIPAA minimum-necessary scripts, TCPA suppression of reassigned numbers

Equity

Cost-based models under-call Medicaid needs; impactability adds access barriers; audit ranks by subgroup

Adoption

Overlay inside the existing CM system; learnable in 1 day; supervisors see time saved, not surveillance

Pilot with one regional plan in 90 days; price a seat at ~1/3 of the time it frees

Brownie points: buyer, pilot scope and pricing signal. Assumptions flagged; to be validated with a design-partner plan

A. WHO BUYS, WHO USES, WHO BLOCKS

Economic buyer

VP Care Management / CMO: owns Stars, HEDIS (TRC, FUH), readmissions and the CM headcount budget

Champion

CM directors and supervisors: feel caseload spikes and audit stress daily

Gatekeepers

CIO/CISO (HITRUST, which HiLabs holds), Compliance (NCQA, Part 2, TCPA), clinical governance for AI

First target

Regional plan with MA + D-SNP + Medicaid, 300k–800k members, an existing HiLabs MCheck customer, running Jiva or GuidingCare

B. PILOT: 90 DAYS, 20 CASE MANAGERS

Day 0–30 · Instrument

Read-only overlay; ingest ADT, pharmacy (incl. rejects), CCD, eligibility, provider directory, CM notes. 2-week time-motion baseline. 10 CMs workbench, 10 matched controls (TOC + BH programs).

Day 31–60 · Shadow AI

Queue + 360 live. AI brief and med diff in shadow mode: clinicians rate accuracy; no drafts saved yet.

Day 61–90 · Assist

Pre-fill + note drafting on, clinician signs. Measure north star, clocks met, after-hours notes, guardrails.

Go/no-go at day 90: ≥ +1.5 conversations/CM/day vs control, ≥ 85% discharges contacted in 2 business days, AI-brief error rate < 5% (rated), CM NPS +20.

C. PRICING SIGNAL

$750

per CM seat per month (~$9k / yr)

Value per CM per year	
Time freed (2 h/day, half the model)	2 h
RN loaded cost (BLS median $93.6k × 1.4)	~$63/h
× 230 working days	≈ $29k
Capacity ROI on time alone	**~3.2×**

Upside not priced in: each avoided readmission ≈ $15.2k; TRC/FUH feed Stars (4+ stars brings a 5% benchmark bonus). Alternative: $0.10–0.25 PMPM on CM-eligible members. Pilot fee $75–100k, credited to Year 1.

Expand: more programs → UM nurses → provider-delegated CM (ACOs)

Appendix: sources, assumptions and how to try the prototype

1. HiLabs Product Caselet 2026–27 (provided): workload ranges, narrative, deliverables

2. CMS ADT CoP FAQ, 42 CFR 482.24(d): minimum notification content, diagnosis optional

3. Kripalani et al., JAMA 2007 (via JABFM 2017): discharge summary at 12–34% of first visits

4. Systematic review of post-discharge medication discrepancies (PMC7235049): median ~50%

5. NBER w34191, Medicaid outreach field experiment: ≥ 24% inaccurate phone numbers

6. CMS OMH Z-code data highlights (2017–2019): Z codes on ~1.1–1.6% of beneficiaries

7. AHRQ HCUP Statistical Brief 248: Medicare 30-day readmission ~17%; avg readmission cost $15.2k (2018)

8. KFF, Medicare Advantage in 2026: MA ≈ 55% of eligibles; SNPs 8.2M (78% D-SNP)

9. KFF, Medicaid and pregnancy: Medicaid finances 41% of US births

10. CMS-0057-F fact sheet: Provider Access, Payer-to-Payer, PA APIs by 1 Jan 2027

11. NCQA Case Management Accreditation standards: file review of initial assessment

12. CA DHCS D-SNP reporting specs: HRA and ICP within 90 days of enrollment

13. HEDIS TRC plan tip sheets (e.g. MVP Health Care 2025): four TRC components

14. CMS MA AI FAQ, Feb 2024 (Norton Rose Fulbright summary); California SB 1120 (2024)

15. Becker's / US Surgeon General: nurses spend up to 41% of the day in the EHR

16. AMN Healthcare 2025 RN survey (58% burnout); AMIA survey (77% after-hours admin work)

17. Care managers & multiple HIT applications (PMC6557451); Wolters Kluwer (3–5 apps per CM)

18. KLAS Best in KLAS 2025 payer care management; HealthEdge & Innovaccer AI announcements (vendor claims)

19. HiLabs: Series B ($39M, 2024), MCheck suite, CAQH partnership (Fierce Healthcare, GlobeNewswire)

20. SNOMED International & Regenstrief LOINC Ontology (2025); HL7 FHIR-to-OMOP IG code mappings

21. van Walraven et al., CMAJ 2010: LACE index (C-statistic 0.68)

22. NLM Clinical Tables (ICD-10-CM, LOINC) and RxNav (RxNorm/NDC) APIs: all annex codes verified 7 Nov 2026

23. BLS Occupational Employment & Wage Statistics: registered nurse median pay

TRY THE PROTOTYPE

QR code to the live prototype (added after Vercel deploy)

URL added after deployment

Key assumptions

Day model inputs (annex, blue cells) · 50% of modelled time captured in the pilot · seat price · baseline TRC 2-day contact ~50% · all members, providers, pharmacies and the plan are fictional; 555-01xx numbers are reserved fictional numbers.